Agenda Item: Closing the Global Mental Health Treatment Gap

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1. Letter From Head of Academy

Dear Delegates,

First of all, I would like to say that I am honored to welcome you all to DALMUN'26. As the Head of Academy, my team and I look forward to hearing about the bright ideas and creative solutions that you will bring to this conference, and especially to this committee. As delegates of the WHO committee, each and every one of you has the responsibility to address global challenges, exchange ideas, and work towards creating effective and sustainable solutions. You have the opportunity to step into the shoes of the leaders who make important decisions around the world. Throughout this committee, you will be expected to approach complex issues from different perspectives while working towards meaningful solutions.

Mental health has become an increasingly important global health concern, affecting individuals, communities, and healthcare systems across the world. Despite growing awareness, many people still face significant barriers to accessing appropriate mental health support and care. Addressing this crisis requires more than national healthcare policies; it requires international cooperation, accessible healthcare systems and public awareness.

I encourage each of you to research your country's position thoroughly, understand the existing international frameworks concerning global mental health, and consider both the challenges and opportunities presented by different approaches. As you develop your solutions, remember the importance of ensuring equal access to healthcare, protecting human dignity, addressing social and economic inequalities, and strengthening international cooperation. Most importantly, remember that Model United Nations is not simply about defending a position; it is about diplomacy, cooperation, and finding common ground.

On behalf of the academy, I wish you all an engaging, productive, and memorable committee. I look forward to witnessing the diplomacy, creativity, and leadership you bring to the floor.

Best regards,

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Head of Academy

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2. Letter from the Under-Secretary-General of WHO

Honourable Delegates,

It is my great pleasure to welcome you to the World Health Organization Committee of DALMUN'26. As your Under-Secretary-General, I am very happy to guide you through an agenda that focuses on one of the most important public health issues of our time, mental health.

I would also like to remind you that mental health is a sensitive topic that affects millions of people. Please approach the subject with the care and respect it deserves throughout the conference.

As your Under Secretary General, I will do my best to support you during the committee and help make sure that both the academic and procedural parts of the conference run smoothly. If you have any questions about the agenda item, the Study Guide, or the Rules of Procedure, please feel free to contact me or a member of our team.

Wishing you all a creative and memorable MUN experience.
Sincerely,

İrem ÇELİK
The Under Secretary General of WHO

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3. Introduction To the Committee

The World Health Organization (WHO) is the United Nations agency that works on international public health. Since it was founded, WHO has brought Member States together to discuss health problems, share information, and work on solutions. Its work covers many different areas, including health emergencies, health systems, and mental health.

The WHO committee gives delegates the chance to discuss health problems from a global perspective. WHO requires delegates to think about things such as health-care systems, access to medical services, scientific evidence, and the different situations of countries. Health is also affected by social, economic, legal, and cultural factors, so cooperation between countries is an important part of the committee.

Before discussing solutions to the global mental health treatment gap, delegates should have a basic understanding of how WHO works. Knowing the Organization's responsibilities, decision-making process, and the role of Member States will help delegates understand what can realistically be done through WHO. The following sections will briefly explain WHO, its structure, and the role of its Member States.

3.1. What is the World Health Organization (WHO)?

The World Health Organization (WHO) was founded in 1948 and is the United Nations agency responsible for international public health. It works with 194 Member States on different health issues, including diseases, health systems, emergencies, and mental health.

WHO helps improve access to health care, collects health data, and supports countries during health emergencies. In mental health, it encourages countries to include mental health services in their health systems.

For this committee, delegates should understand what WHO can realistically do. Solutions should focus on cooperation between countries while respecting their different systems and needs.

3.2. Structure and decision-making process of WHO

There are three main bodies in the World Health Organization; the World Health Assembly (WHA), the Executive Board, and the Secretariat. WHA is the principal decision-making organ that includes all Member States. Its meetings take place annually to discuss matters relating to health, budget approval, and decisions. Each member state has one vote in the meeting. The role of the executive board is to assist in preparing the agenda of the WHA and executing its decisions.

The secretariat of the organization is headed by the Director-General. The role of the Secretariat is to undertake daily management functions of WHO and assist member states. Due to differences in health systems and priorities of countries, there might be disagreements among them.

3.3. Role of Member States in WHO

Member States play an important role in the work of the World Health Organization. They participate in discussions, vote on decisions, and cooperate with each other to address health problems that affect both their own populations and the international community.

In the WHO committee, delegates should research their country's existing mental health policies, health care system, and position on the agenda item.

4. Introduction To the Agenda Item

Mental health is an important part of people's health, but many people around the world still cannot get the care they need. The problem is very serious in low income countries, where mental health services are limited to access. Stigma can also stop people from asking for help. Children and young people may face challenges because of school pressure, social media, and limited access to support.

4.1. Key Terms and Concepts

Mental disorder: An illness that impacts a person's mind, body, and actions. Examples are anxiety, depression, bipolar disorder, and schizophrenia.

Treatment gap: The disparity between those in need of mental health services and those who get them.

Stigma: Negative beliefs and perceptions about mental health patients.

Task-shifting: The practice of having untrained mental health providers delivering basic mental health services when there is a shortage of trained professionals.

Universal Health Coverage (UHC): A system that ensures that all individuals can access necessary health services without worrying about costs. This also includes mental health services.

Psychosocial disability: The condition in which a patient's ability to engage in normal activities is impaired due to mental health problems.

5. Background on the Agenda Item: Global Mental Health, Human Rights, and Health System Integration

Mental health represents one of the most critical yet historically neglected pillars of global public health. Worldwide, hundreds of millions of people live with mental, neurological, and substance use disorders, facing profound health, social, economic, and human rights consequences. As the leading international authority on health, the World Health Organization (WHO) works to promote mental well being, protect vulnerable populations, and ensure that quality mental healthcare is accessible to all. Despite significant progress in medical science, systemic disparities, severe underfunding, structural stigma, and outdated legal frameworks continue to restrict access to essential psychosocial support. Addressing these complex

challenges is essential to fulfilling the global commitment to Universal Health Coverage (UHC), advancing human rights, and achieving Sustainable Development Goal 3 (SDG 3).

5.1. Systemic Integration and Community Based Care

Integrating mental healthcare into primary healthcare systems and community networks is a vital strategy for addressing the global burden of mental disorders. Historically, mental healthcare has been heavily centralized within specialized psychiatric facilities, leaving rural, low income, and marginalized populations without access to essential support. Over 80% of individuals with mental health conditions in low and middle-income countries (LMICs) receive no formal treatment. The WHO advocates for decentralized models where primary health practitioners, clinical nurses, and community health workers are trained to deliver baseline psychosocial interventions. Building local workforce capacity through non-specialist training bridges the treatment gap, reduces institutional dependence, and ensures that mental healthcare is embedded directly within local communities.

5.1.1 Task Sharing and Non-Specialist Capacity Building

Task-sharing involves the rational redistribution of healthcare tasks among health workforce teams to expand coverage in resource constrained settings. Given the severe global shortage of specialized psychiatrists and psychologists, primary care workers and community volunteers can be trained to screen, manage, and refer to mental health conditions. Specialist personnel from tertiary facilities transition into supervisory roles, equipping frontline workers with evidence based psychosocial toolkits. This approach decentralizes care, reduces operating costs, and fosters culturally appropriate care delivered by trusted local figures.

<u>China</u>

China is one of the world's largest nations and exerts a significant influence on global mental health. According to WHO data regarding China, approximately 54 million people suffer from depression, while 41 million have anxiety disorders. Although mental health services in China have expanded in recent years, serious challenges persist. While there is a substantial need for psychological and psychiatric services across the country, access to comprehensive care remains unequal. Shortages in psychiatric resources and personnel, combined with regional disparities, contribute to the chronicity of these conditions. Beyond these specific issues, emerging psychological problems are causing widespread distress—often manifesting as sleep disturbances—across the nation.

Mental health issues among young people have also become a growing concern. Factors such as academic pressure, societal expectations, intense family pressure, employment challenges, and rapid social change can exacerbate psychological problems. Furthermore, the social stigma surrounding mental health issues can deter individuals from seeking professional help.

To address these concerns and improve the situation, China has designated the years 2025–2027 as the "Year of Pediatric and Mental Health Services" in an effort to combat rising mental health issues and sleep-related problems. Under the leadership of the National Health

Commission (NHC), the goal is to establish a permanent and comprehensive psychological crisis intervention system nationwide by 2030. This intervention network includes: A unified national mental health and psychological support hotline (12356) with easily accessible information on its scope was launched; the expansion of sleep and mental health outpatient clinics to at least one hospital in every province was mandated; and screening and prevention measures in schools were strengthened to address rising cases of anxiety among children and adolescents.

Türkiye

Türkiye stands at the forefront of nations facing both significant progress and serious structural challenges in the realm of global mental health. Although mental health services have been integrated into the healthcare system, issues such as access to services, a shortage of specialists, regional disparities, and social stigma persist. Furthermore, misconceptions about psychiatry within society, erroneous perspectives held by families, and adverse events leading to misdiagnoses profoundly impact individuals and the community at large.

One of the most critical issues in Türkiye is the gap between the demand for mental health services and existing capacity. While psychiatric services are more accessible in major cities, obtaining comprehensive data and reaching specific, often underserved groups remains difficult; a person's likelihood of accessing mental health care is heavily influenced by their geographic location.

Simultaneously, the social landscape in Türkiye is marked by issues such as academic pressure, excessive family expectations, and bullying—whether perpetrated by teachers or peers within the school environment. Workplace mobbing and bullying among colleagues serve as further examples. Such incidents contribute to significant adverse consequences across the country.

Currently, there is no legislation in Türkiye that provides holistic support for mental health. Despite a comprehensive Mental Health Bill being presented to parliamentary bodies over many years, it has yet to be enacted into law. Meanwhile, the long-term management of chronic mental health conditions is conducted within the framework of guidelines established for Community Mental Health Centers (TRSM).

5.1.2. Bridging the Treatment Gap in Low and Middle-Income Countries (LMICs)

The mental health treatment gap in LMICs remains a major barrier to equal public health. Weakening in health infrastructure, severe budget constraints, and the concentration of specialists in urban centers leave low income populations largely underserved. Strengthening primary healthcare networks with basic diagnostic tools, essential psychotherapy medications, and counseling services enables health systems to catch early stage conditions. Through localized clinics and mobile health teams, member states can expand access to essential care without forcing patients to travel long distances or incur catastrophic out of pocket expenses.

5.2. Youth and Workplace Mental Health in the Digital and Modern Era

Rapid socio economic shifts, digital hyper connectivity, and changing labor dynamics have introduced complex mental health challenges for youth and working populations. Rates of anxiety, depression, and psychological distress among young people have escalated significantly, fueled by social media pressures, cyberbullying, academic stress, and economic uncertainty. Concurrently, workplace stress, unsafe labor conditions, long hours, and the expansion of the gig economy contribute to high rates of professional burnout and depression. Addressing these modern drivers of psychological strain requires coordinated policies that balance technology use, safeguard worker rights, and integrate mental health promotion into educational and employment systems.

Japan

Despite being a country that is far ahead in terms of advanced healthcare interventions and medical institutions, Japan faces serious and complex mental health challenges. In contrast to the country's robust healthcare infrastructure, issues such as rejected diagnoses, late detection of mental illnesses, and a lack of societal acceptance are prevalent.

One of the fundamental issues in Japan is the intense work culture and societal pressure. Long working hours extending late into the night, high professional expectations, and fierce workplace competition generate significant stress and psychological burnout for many individuals. Given that the adult population constitutes the majority of society, the stress they shoulder deeply affects their children as well.

Another major issue in Japan is social isolation. The elderly population is growing, many people live alone, and some individuals remain detached from society for extended periods. This situation hinders communication and weakens social bonds. When people feel little closeness to one another, mental health issues may go unnoticed for longer, potentially causing the situation to deteriorate further.

Japan has established regulations and laws to address various psychological disorders. With the Kōnin Shinrishi Law (Certified Public Psychologist Act)—passed in 2015 and enacted in 2017/2018—Japan granted legal status to psychology and mental health services. The Mental Health and Welfare Act is a foundational law governing the medical care, protection, and social reintegration of individuals with mental disorders or disabilities. Japan introduced its first regulation in this field in 1900 and has since updated it through modern reforms.

United States

Despite possessing a mental health services network, the United States faces a serious mental health crisis. Key weaknesses in the system include rising mental health issues—particularly among young people—inequities in access to services, and high treatment costs. The U.S. Centers for Disease Control and Prevention (CDC) also views the state of mental health in the country, especially among youth, as a major public health issue. A primary underlying cause of these problems is the lack of a cohesive legal framework.

The cost of mental health services has become a significant issue for many people in the U.S. Obtaining treatment and psychological support is often very expensive, and even health insurance frequently fails to cover all needs. Consequently, some individuals are forced to delay seeking help, and those in need of mental health care may lose valuable time trying to find professional support.

Youth mental health is also a major concern. School-based violence negatively impacts the mental well-being of both students and teachers, subjecting individuals to severe trauma associated with schools.

Currently, the United States lacks a comprehensive body of legislation specifically regulating psychological services beyond professional licensing requirements; however, school-based counseling services have been expanded and strengthened to adequately support children who otherwise lack access to psychological care.

5.2.1. Youth Mental Health and Digital Ecosystems

The digital era has transformed how young people interact, learn, and socialize, offering opportunities for connection alongside significant psychological risks. Continuous exposure to online comparison, cyberbullying, and addictive platform designs contributes to heightened distress, body image issues, and sleep deprivation among adolescents. While digital platforms can serve as avenues for tele-mental health support, comprehensive regulatory frameworks, digital literacy education in schools, and platform accountability standards are essential to shield youth from online harms while expanding access to safe, evidence based digital care.

5.2.2. Workplace Stress, Burnout, and the Gig Economy

Workplace environments exert a direct influence on mental well being and overall economic productivity. Prolonged work hours, job insecurity, and lack of mental health protections strain employee well being, driving up absenteeism and burnout rates. The rise of remote work and platform based gig labor has further blurred the boundaries between professional and personal life, frequently isolating workers from traditional labor protections or employer sponsored health benefits. The WHO emphasizes the need for occupational health policies that enforce reasonable working hours, promote supportive workplace cultures, and extend comprehensive mental health benefits to all categories of workers.

5.3. Human Rights, Decriminalization, and Suicide Prevention

Protecting the fundamental human rights of individuals with mental health conditions is essential for building ethical and effective health systems. In many jurisdictions, outdated legal frameworks permit involuntary confinement, mechanical restraint, forced medication, and isolation within psychiatric institutions. Furthermore, suicide attempts remain criminalized in several countries, creating fear and stigma that prevent vulnerable individuals from seeking emergency assistance. Aligning national legislation with international frameworks—such as the UN Convention on the Rights of Persons with Disabilities

(CRPD)—is critical to dismantling coercive institutional models, advancing rights based care, and scaling up evidence based suicide prevention strategies.

5.3.1. Transitioning to Rights Based, Non-Coercive Care (CRPD Alignment)

The CRPD establishes that individuals with psychosocial disabilities possess legal capacity on an equal basis with others, challenging traditional non-consensual commitment and treatment practices. To uphold dignity and personal autonomy, health systems must transition from substituted decision-making models to supported decision-making structures. This transition involves eliminating involuntary decisions in favor of voluntary, community-based care, supported housing, and crisis resolution services that honor individual choices and promote social inclusion.

5.3.2. Suicide Decriminalization, Harm Reduction, and Crisis Strategies

Suicide claims over 700,000 lives annually, disproportionately impacting young adults and marginalized communities. Criminalizing suicide attempts exacerbates psychological trauma and deters individuals from seeking urgent medical and psychological help. Effective national suicide prevention strategies rely on harm reduction principles, including the decriminalization of suicide attempts, the establishment of 24/7 crisis helplines, restricting access to lethal means, public awareness campaigns to reduce stigma, and integrated crisis response teams equipped to de-escalate emergency situations safely.

United Kingdom

The United Kingdom is internationally recognized for having a highly effective public health system regarding mental health services. However, there is a significant gap between perceptions and reality concerning mental health. This stems from the fact that not everyone in the country has access to these services.

The primary challenge facing mental health services in the UK relates to the imbalance between supply and demand. Although these services exist, access is hindered by long waiting times, staff shortages, and high demand. While early diagnosis of mental health disorders is crucial, the core issue is the inability to secure these diagnoses in a timely manner.

The fact that mental health services are free of charge is often viewed as an advantage of the UK's mental health sector. However, being free does not always equate to being accessible. If an individual cannot receive appropriate help because they are forced to wait months or because there are insufficient specialists in their area, the fact that the services are free becomes irrelevant. Consequently, the UK's claim of providing "accessible healthcare" is highly questionable.

The UK must first address its own challenges regarding accessibility and capacity building before attempting to influence how other countries manage their mental health services. For the international community, what matters is not merely that the UK possesses a well-developed health system, but also how quickly and efficiently it can deliver services.

5.4. Emergency Response, Crisis Support, and Environmental Determinants

Populations living through armed conflict, natural disasters, forced displacement, and environmental degradation experience high rates of trauma and prolonged distress. Despite these heavy burdens, mental health and psychosocial support (MHPSS) remains underfunded in emergency responses compared to physical relief such as food, shelter, and basic sanitation. Furthermore, chronic environmental instability and severe weather events associated with climate change drive rising levels of "eco-anxiety" and distress, particularly in vulnerable communities. Integrating MHPSS into emergency response protocols and national climate adaptation plans is vital for building long term psychological resilience.

5.4.1. Psychosocial Support in Conflict Zones and Humanitarian Crises

Humanitarian emergencies inflict acute psychological distress, disrupt community support networks, and exacerbate pre-existing mental health conditions. Delivering effective MHPSS in crisis settings requires adhering to inter-agency standards such as the Inter-Agency Standing Committee (IASC) Guidelines which prioritize psychological first aid, community based support, and specialized clinical care. Securing dedicated funding for mental health within humanitarian appeal budgets ensures that psychological care is delivered alongside physical aid, helping affected populations rebuild their lives and communities.

5.4.2. Climate Change, Environmental Degradation, and Eco-Anxiety

The growing frequency of climate driven disasters, extreme weather, and environmental displacement imposes severe psychological strains on affected populations. Beyond acute disaster trauma, the ongoing destruction of local ecosystems creates persistent uncertainty, loss of a productive life, and climate induced distress, commonly termed eco-anxiety. Member states must integrate mental health considerations directly into national climate adaptation strategies, disaster risk reduction plans, and community response frameworks to protect vulnerable populations facing environmental disruption.

South Korea

While South Korea is one of the most technologically advanced nations in the world, there are still a number of severe issues when it comes to the sphere of mental healthcare in the country. The legislation of this issue is regulated by the Mental Health Welfare Act that ensures legal protection to people with mental illnesses, their treatment and rehabilitation, as well as prevention of discrimination against them. Furthermore, this law emphasizes the significance of providing protection to children and adolescents with their early diagnostics and counseling. However, the existence of this legal act does not mean that everyone can use it equally.

There also exists similar legislation in South Korea pertaining to mental well-being and emergency care. The Suicide Prevention and Promotion of Life Respecting Culture Act has

been the basis of legal policy formulation for all policies related to suicide prevention, counseling, crisis intervention, creating public awareness, and coordination among stakeholders. The objective of all these policies is to create a culture of life respecting and to motivate people to seek professional help in case of difficulties pertaining to mental health. However, legislation alone can neither dispel the stigma attached with these problems nor ensure prompt help.

As such, the mental health law in South Korea covers areas including prevention, early intervention, treatment, rehabilitation, protection against discrimination, and crisis intervention. South Korea will continue to evolve in terms of these legal and institutional structures, especially for youth and communities. However, effective implementation will be very important. In order to make sure that these legal rights and services are well implemented, it is imperative for South Korea to ensure the availability of services, proper capacity building among professionals, and efforts at reducing stigma. The South Korea example shows that comprehensive laws can provide some important protections; however, it will all come down to their implementation.

France

Despite having an effective healthcare system, the way France utilizes its mental health system warrants critical examination. The government has designated mental health as the "Grande Cause Nationale" (Great National Cause) for 2025 and is planning a new psychiatric strategy comprising three key pillars and twenty-six prevention measures. Indeed, there is an acknowledgment that the French healthcare system faces challenges regarding psychiatric capacity and resources.

Social inequalities present another hurdle in the delivery of mental health services in France. While efforts are being made to minimize disparities in treatment, it is recognized that gaps persist—particularly concerning timely medical care, diagnosis, and access to services. The government is also implementing programs aimed at destigmatizing mental health issues, such as mental health first aid training. Thus, the discussion in France goes beyond merely describing the state of healthcare; it also addresses why equal access to mental health services remains elusive.

5.5. Vulnerable Populations: Lifespan and Specialized Care

Addressing mental health across the lifespan requires targeted strategies for specific vulnerable groups, notably mothers during pregnancy and aging populations facing neurodegenerative conditions. Perinatal depression affects approximately 1 in 5 women during pregnancy or the first year after childbirth, negatively affecting maternal well being and early child development. Simultaneously, rising global life expectancies have increased the prevalence of late life depression, social isolation, and neurodegenerative disorders such as Alzheimer’s disease. Building inclusive health systems requires embedding routine screenings into maternal care and expanding specialized long term infrastructure for elderly populations.

6. Existing International Frameworks and WHO Programs

The existing international framework and WHO programs pertain to the global strategies and initiatives that have been created to combat the problems related to mental health care and minimize the divide between individuals who require mental health care services and those who have access to them. Such international frameworks strive to enhance the availability, accessibility, and quality of mental health care services and encourage countries to upgrade their health care systems.

6.1. WHO Mental Health Action Plan 2021–2030

The WHO Mental Health Action Plan 2021–2030 is a framework designed to improve mental health care around the world. It generally focuses on increasing access to affordable and quality mental health services. It also works to protect the rights of people with mental health conditions. The plan supports integrating health care into national health systems. It encourages countries to develop policies and services. The goal is to reduce the mental health treatment gap.

6.2. WHO Mental Health Gap Action Programme (mhGAP)

mhGAP is an abbreviation for the WHO's Mental Health Gap Action Programme that was developed because of the problem of inadequate mental health service provision, particularly in low- and middle-income nations. This program provides assistance to nations in training general health workers on how to offer basic care to people with mental disorders using the current health infrastructure.

6.3. mhGAP Intervention Guide (mhGAP-IG)

MhGAP Intervention Guide is a useful resource developed by WHO which assists health practitioners without special training to recognize and provide interventions for some common mental, neurological and substance use conditions. The resource contains evidence-based guidance on assessment, intervention, and management of the above conditions. It assists countries to scale up essential mental health services in their countries.

7. Topics a Resolution Should Address

1. How can Mental Healthcare be integrated into Primary Health Systems and Community Networks?
2. What Precautions Should Be Taken to Protect Youth Mental Health in the Digital Age?
3. How Can The Absence of Psychosocial Support in Conflict Zones, Disasters and Humanitarian Emergencies Be Combatted?
4. How can Member States strengthen workplace mental health protections and ensure fair working conditions for gig and remote workers?
5. How can Member States strengthen suicide prevention strategies by improving access to crisis support, reducing stigma, and ensuring individuals in crisis can seek help without fear of legal consequences?
6. What measures can Member States implement to address eco-anxiety and other mental health impacts caused by climate change and environmental disasters?
7. How can Member States protect the human rights of people with mental health conditions?
8. How can Member States reduce the mental health treatment gap in LMICs?
9. How can task-sharing improve access to mental healthcare?
10. How can Member States support vulnerable groups across different stages of life?

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